



AbaTools

ADHD SCREENING PACK

ADHD Screening Scales & Protocol

A structured library of **20+ ready-to-use ADHD screening scales** for pediatric assessment — organising parent, teacher and clinical observations into clear, defensible screening decisions.



Assessment • Screening • Interpretation • Referral

22

Scales, screens & worksheets

11

Ready-to-use rating forms

3–17

Pediatric age range

HOW TO USE THIS PACK

Before You Begin

This pack centralises the most useful pediatric ADHD screening scales into a single, consistent workflow — so you can gather, organise and interpret observations with more confidence.



What this pack is

A library of structured screening scales and worksheets that help you capture inattention, hyperactivity–impulsivity and functional impact across home and school.



How it is organised

Each scale states its purpose, respondent, age range and how it is scored. Use one, several, or the full protocol depending on the referral question.



What it is not

These are **screening–support** tools. They flag risk and organise information — they do **not** confirm a diagnosis or replace a comprehensive clinical evaluation.



Professional use

Interpretation requires appropriate training and clinical judgement. Always corroborate scores with history, direct observation and multiple informants.

A RELIABLE SCREENING SEQUENCE

- 1 Clarify the referral question**
Define the concern, the settings involved and who can provide reliable observations.
- 2 Select the relevant scales**
Use the Master Library Index to choose parent, teacher and domain-specific screens.
- 3 Gather multi-informant input**
Collect at least one home and one school report; consistency across settings matters.
- 4 Score, interpret & decide next steps**
Total each scale, weigh functional impairment, then document and route the case.

MASTER LIBRARY

Scale Index — Part 1

Twenty-two screening scales, checklists and worksheets. Choose by respondent, age and the domain you need to evaluate. Every entry is ready to print and use.

#	SCALE / TOOL	FORMAT	RESPONDENT	AGE	ITEMS	PRIMARY FOCUS
01	Parent / Caregiver Inattention Screen	Form	Parent	4–17	12	Attention, focus, follow-through, organisation
02	Parent / Caregiver Hyperactivity–Impulsivity Screen	Form	Parent	4–17	12	Motor activity, restlessness, impulse control
03	Teacher Classroom Observation Screen	Form	Teacher	4–17	14	Attention & behaviour in the school setting
04	Combined Presentation Quick Screen	Screen	Any	6–17	8	Rapid combined-symptom triage
05	Cross-Setting Consistency Screen	Screen	Clinician	4–17	7	Agreement of symptoms across settings
06	Executive Function & Organisation Screen	Screen	Parent / Teacher	6–17	8	Planning, working memory, task initiation
07	Emotional Regulation & Frustration Screen	Screen	Parent / Teacher	4–17	7	Reactivity, frustration tolerance, mood shifts
08	Preschool / Early–Years Behaviour Screen	Screen	Parent / Carer	3–5	8	Early activity & attention indicators
09	Homework, Study & Academic Behaviours	Screen	Parent / Teacher	6–17	7	Academic engagement & task completion
10	Social & Peer Interaction Screen	Screen	Parent / Teacher	5–17	7	Peer relationships & social impulsivity
11	Sleep, Activity & Daily Routine Screen	Screen	Parent	4–17	7	Sleep, routine & regulation factors



The pack works as a **modular protocol**. For a focused question use a single scale; for a full work-up combine parent, teacher and domain screens, then consolidate on the Screening Summary (Scale 21).

MASTER LIBRARY

Scale Index — Part 2

Differential-flag checklists and clinical worksheets complete the protocol — supporting rule-out reasoning, history-taking, monitoring and documentation.

#	SCALE / TOOL	FORMAT	RESPONDENT	AGE	ITEMS	PRIMARY FOCUS
12	Oppositional / Conduct Differential Flags	Checklist	Parent / Teacher	5–17	8	Rule-out: defiance & conduct concerns
13	Anxiety & Mood Differential Flags	Checklist	Parent / Clinician	6–17	8	Rule-out: internalising overlap
14	ASD–Overlap Differential Flags	Checklist	Clinician	4–17	8	Rule-out: social-communication overlap
15	Strengths & Protective Factors Inventory	Worksheet	Parent / Teacher	4–17	—	Resilience, interests & supports
16	Developmental & Medical History Screen	Worksheet	Parent	Any	—	Onset, milestones, medical context
17	Parent Interview Prompt Guide	Worksheet	Clinician	Any	—	Structured caregiver interview cues
18	Teacher Narrative & Work-Sample Request	Worksheet	Teacher	5–17	—	Qualitative classroom evidence
19	Baseline Monitoring & Target-Behaviour Log	Worksheet	Any	Any	—	Frequency baseline before/after change
20	Functional Impairment & Daily-Life Screen	Screen	Parent / Teacher	4–17	8	Impact on home, school & relationships
21	Screening Summary & Severity Consolidation	Worksheet	Clinician	Any	—	Bring all scores together
22	Referral & Next-Step Decision Worksheet	Worksheet	Clinician	Any	—	Document triage & recommendations

• **Form**
Full printable rating scale

• **Screen**
Compact fillable scale

• **Checklist**
Differential / rule-out flags

• **Worksheet**
Interview, log & documentation

CLINICAL REFERENCE

Understanding ADHD

ADHD is described across two symptom dimensions and three clinical presentations. Screening aims to capture both – across more than one setting – alongside real functional impact.



Dimension 1 – Inattention

Difficulty sustaining attention, careless errors, not following through, disorganisation, avoiding effortful tasks, losing items, distractibility and forgetfulness.



Dimension 2 – Hyperactivity–Impulsivity

Fidgeting, leaving the seat, restlessness, difficulty playing quietly, "on the go", excessive talking, blurting answers, difficulty waiting and interrupting.

THREE CLINICAL PRESENTATIONS

PRESENTATION	SCREENING PICTURE
Predominantly Inattentive	Inattention symptoms predominate; activity level may appear typical. Often under-identified.
Predominantly Hyperactive–Impulsive	Motor and impulsive symptoms predominate; more common in younger children.
Combined	Both dimensions are clearly present and impairing across settings.



Signal to look for: symptoms that are **developmentally excessive**, present **before age 12**, evident in **two or more settings**, and causing **clear functional impairment**. A single high score in one setting is a prompt to gather more information – not a conclusion.

S-01 Parent Inattention Screen

Parent / Caregiver report · Ages 4–17

INATTENTION DIMENSION

▶ CLIENT & SCREENING INFORMATION

Child / Client:

Age:

Completed by:

Date of birth:

Grade / Setting:

Relationship / Role:

Date:

▶ INSTRUCTIONS

Think about the child's behaviour over the **last 6 months** compared with other children of the same age. Mark how often each behaviour occurs.

▶ SCORING KEY

0

Never / Rarely

Not present

1

Sometimes

Mild / occasional

2

Often

Clearly present

3

Very Often

Marked / pervasive

SCREENING ITEMS

#	BEHAVIOUR / OBSERVATION	0	1	2	3
1	Fails to give close attention to detail or makes careless mistakes in schoolwork or activities	☐	☐	☐	☐
2	Has difficulty sustaining attention during tasks, play or conversation	☐	☐	☐	☐
3	Does not seem to listen when spoken to directly	☐	☐	☐	☐
4	Does not follow through on instructions and fails to finish tasks or chores	☐	☐	☐	☐
5	Has difficulty organising tasks, belongings and activities	☐	☐	☐	☐
6	Avoids or strongly dislikes tasks that require sustained mental effort	☐	☐	☐	☐
7	Loses things needed for tasks (toys, pencils, books, devices, assignments)	☐	☐	☐	☐
8	Is easily distracted by surrounding sights, sounds or unrelated thoughts	☐	☐	☐	☐
9	Is forgetful in daily routines and activities	☐	☐	☐	☐
10	Shifts from one unfinished activity to another	☐	☐	☐	☐
11	Needs frequent reminders or supervision to complete age-typical tasks	☐	☐	☐	☐
12	Struggles to keep track of time, deadlines or multi-step routines	☐	☐	☐	☐

S-02

Parent Hyperactivity–Impulsivity Screen

Parent / Caregiver report · Ages 4–17

HYPERACTIVITY–IMPULSIVITY DIMENSION

▶ CLIENT & SCREENING INFORMATION

Child / Client:

Date of birth:

Age:

Grade / Setting:

Date:

Completed by:

Relationship / Role:

▶ INSTRUCTIONS

Compared with other children of the same age, mark how often each behaviour has occurred over the **last 6 months**.

▶ SCORING KEY

0

Never / Rarely

Not present

1

Sometimes

Mild / occasional

2

Often

Clearly present

3

Very Often

Marked / pervasive

SCREENING ITEMS

#	BEHAVIOUR / OBSERVATION	0	1	2	3
1	Fidgets with hands or feet, or squirms in seat	☐	☐	☐	☐
2	Leaves seat when remaining seated is expected	☐	☐	☐	☐
3	Runs about or climbs in situations where it is inappropriate (or feels restless)	☐	☐	☐	☐
4	Has difficulty playing or engaging in activities quietly	☐	☐	☐	☐
5	Is "on the go" or acts as if "driven by a motor"	☐	☐	☐	☐
6	Talks excessively	☐	☐	☐	☐
7	Blurts out answers before questions are completed	☐	☐	☐	☐
8	Has difficulty waiting for a turn	☐	☐	☐	☐
9	Interrupts or intrudes on others' conversations or games	☐	☐	☐	☐
10	Acts without thinking about consequences	☐	☐	☐	☐
11	Has difficulty slowing down or settling, even when tired	☐	☐	☐	☐
12	Grabs, touches or reacts impulsively to objects or people	☐	☐	☐	☐

S-03

Teacher Classroom Observation Screen

Teacher / School-staff report · Ages 4–17

SCHOOL SETTING

▶ CLIENT & SCREENING INFORMATION

Child / Client:
Age:
Completed by:

Grade / Setting:

Date of birth:
Date:
Relationship / Role:

▶ INSTRUCTIONS

Based on your observation of this student in the classroom over the **last 6 months**, rate how often each behaviour occurs relative to peers.

▶ SCORING KEY

0

Never / Rarely

Not present

1

Sometimes

Mild / occasional

2

Often

Clearly present

3

Very Often

Marked / pervasive

SCREENING ITEMS

#	BEHAVIOUR / OBSERVATION	0	1	2	3
1	Has difficulty staying focused during lessons or independent work	☐	☐	☐	☐
2	Makes careless errors or rushes through assignments	☐	☐	☐	☐
3	Is easily distracted by classroom activity or noise	☐	☐	☐	☐
4	Has trouble following multi-step instructions	☐	☐	☐	☐
5	Leaves work incomplete or fails to hand it in	☐	☐	☐	☐
6	Is disorganised with materials, desk or assignments	☐	☐	☐	☐
7	Fidgets, taps or is restless at the desk	☐	☐	☐	☐
8	Leaves seat or moves around at inappropriate times	☐	☐	☐	☐
9	Calls out, blurts answers or interrupts the lesson	☐	☐	☐	☐
10	Has difficulty waiting for a turn in group activities	☐	☐	☐	☐
11	Struggles with transitions between tasks or settings	☐	☐	☐	☐
12	Requires more redirection or supervision than peers	☐	☐	☐	☐
13	Has difficulty with peer interactions during unstructured time	☐	☐	☐	☐
14	Underperforms relative to apparent ability	☐	☐	☐	☐

DOMAIN SCREENS

Symptom & Setting Screens

Use these compact, fillable screens to triage quickly and to confirm that difficulties are present across more than one environment.

Scale 04 — Combined Presentation Quick Screen

Any informant · Ages 6–17 · 8 items

Rapid triage across both symptom dimensions

1	Loses focus quickly on non-preferred tasks	
2	Frequently restless or fidgety	
3	Acts or speaks impulsively	
4	Forgets or misplaces everyday items	
5	Struggles to wait or take turns	
6	Leaves tasks unfinished	
7	Interrupts others often	
8	Requires constant supervision to stay on task	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 24

Scale 05 — Cross-Setting Consistency Screen

Clinician-rated · Ages 4–17 · 7 items

Do symptoms appear in more than one setting?

1	Attention difficulties present at home	
2	Attention difficulties present at school	
3	Hyperactivity/impulsivity present at home	
4	Hyperactivity/impulsivity present at school	
5	Concerns reported by more than one adult	
6	Difficulties evident in unstructured settings	
7	Impact seen in both academic and social areas	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 21

DOMAIN SCREENS

Regulation & Executive Screens

Executive-function and emotional-regulation difficulties frequently accompany ADHD and shape the support a child needs. Screen both to build a fuller picture.

Scale 06 — Executive Function & Organisation

Parent / Teacher · Ages 6–17 · 8 items

Planning, working memory and task initiation

1	Struggles to start tasks without prompting	
2	Has trouble planning multi-step work	
3	Forgets instructions or steps midway	
4	Loses track of belongings and assignments	
5	Has difficulty estimating or managing time	
6	Switches tasks before finishing	
7	Needs help breaking work into steps	
8	Has trouble keeping materials organised	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 24

Scale 07 — Emotional Regulation & Frustration

Parent / Teacher · Ages 4–17 · 7 items

Reactivity, frustration tolerance, mood shifts

1	Becomes frustrated more easily than peers	
2	Has intense reactions to small setbacks	
3	Struggles to calm down once upset	
4	Shows rapid shifts in mood or energy	
5	Gives up quickly when challenged	
6	Reacts impulsively when emotional	
7	Has difficulty coping with changes in plans	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 21

DOMAIN SCREENS

Early-Years & Academic Screens

Screen developmentally appropriate indicators in young children, and the academic behaviours where ADHD most often becomes visible in school-age children.

Scale 08 — Preschool / Early-Years Behaviour

Parent / Carer · Ages 3–5 · 8 items

Early activity and attention indicators

1	Is far more active than other children the same age	
2	Cannot stay with an activity for long	
3	Has difficulty sitting for stories or meals	
4	Climbs or runs excessively, with little caution	
5	Has frequent, intense tantrums	
6	Switches between toys very quickly	
7	Has difficulty following simple routines	
8	Needs constant supervision for safety	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 24

Scale 09 — Homework, Study & Academic Behaviours

Parent / Teacher · Ages 6–17 · 7 items

Academic engagement and task completion

1	Avoids or delays starting homework	
2	Needs frequent reminders to stay on task	
3	Makes careless errors when rushing	
4	Forgets to record or submit assignments	
5	Has difficulty studying for tests	
6	Work quality is inconsistent day to day	
7	Takes much longer than expected to finish work	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 21

DOMAIN SCREENS

Social & Functional-Impact Screens

Symptoms only matter clinically when they impair real life. Screen social functioning and the everyday impact that should weigh heavily in any referral decision.

Scale 10 — Social & Peer Interaction

Parent / Teacher · Ages 5–17 · 7 items

Peer relationships and social impulsivity

1	Interrupts or intrudes during peer play	
2	Has difficulty taking turns in games	
3	Misreads social cues or reacts impulsively	
4	Struggles to keep friendships over time	
5	Is excluded or in frequent conflict with peers	
6	Talks over others or dominates interaction	
7	Has difficulty cooperating in group tasks	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 21

Scale 20 — Functional Impairment & Daily Life

Parent / Teacher · Ages 4–17 · 8 items

Real-world impact across key domains

1	Academic performance is affected	
2	Relationships with family are strained	
3	Friendships and peer standing are affected	
4	Daily routines (morning, bedtime) are difficult	
5	Self-esteem or confidence is affected	
6	Safety incidents occur due to impulsivity	
7	Participation in activities/sports is affected	
8	Significant adult effort is needed to compensate	

Rate each 0–3 (0 Never · 3 Very Often)

Subtotal: / 24

RULE-OUT REASONING

Differential Flags

Several conditions mimic or co-occur with ADHD. Use these flag checklists to decide whether a broader evaluation or a different primary focus is warranted.

▮ SCALE 12 · OPPOSITIONAL / CONDUCT	▮ SCALE 13 · ANXIETY & MOOD	▮ SCALE 14 · ASD OVERLAP
✓ Argues with adults / defies rules ✓ Loses temper frequently ✓ Deliberately annoys others ✓ Blames others for mistakes ✓ Angry, resentful or spiteful ✓ Aggression toward people or property ✓ Pattern is defiant rather than inattentive ✓ Concerns mainly about conduct	✓ Excessive worry or fearfulness ✓ Restlessness driven by anxiety ✓ Sleep difficulty from worry ✓ Low mood or irritability ✓ Loss of interest or motivation ✓ Physical complaints (headaches, stomach) ✓ Attention dips during low mood ✓ Onset linked to a stressor	✓ Marked social-communication difficulty ✓ Restricted or repetitive behaviours ✓ Strong need for sameness ✓ Unusual sensory responses ✓ Limited reciprocal conversation ✓ Difficulty with imaginative/social play ✓ Intense, narrow interests ✓ Concerns present from early childhood



Flags are **not diagnoses**. Multiple flags in a domain suggest that ADHD screening alone is insufficient — consider co-occurrence or refer for a broader assessment. ADHD frequently co-exists with these conditions rather than excluding them.

MAKE SENSE OF THE SCORES

Scoring & Interpretation

Totals organise observations into a defensible screening picture. They indicate the strength of a signal – never a diagnosis on their own.

HOW TO SCORE

- 1

Total each scale

Sum the 0–3 ratings. Each 12-item form ranges 0–36; compact screens are scored out of their own maximum.
- 2

Count "Often / Very Often" responses

Items rated 2 or 3 are **symptom-positive**. Count them within each dimension as a second indicator.
- 3

Compare across informants & settings

Look for agreement between parent and teacher reports. Cross-setting consistency strengthens the signal.
- 4

Weigh functional impairment

High symptom counts **with** clear impairment (Scale 20) carry the most clinical weight.

INTERPRETING THE SCREENING BAND (PER 12-ITEM FORM)

BAND	SCORE RANGE	SUGGESTED SCREENING ACTION
● Low	0–11	Few concerns flagged. Monitor and reassess if difficulties emerge.
● Mild	12–20	Some indicators present. Gather more information and a second informant.
● Moderate	21–28	Clear screening signal. Confirm cross-setting impact; consider full evaluation.
● High	29–36	Strong signal with likely impairment. Refer for comprehensive diagnostic assessment.

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Bands are **interpretive guides for screening**, not validated diagnostic cut-offs. Always integrate scores with developmental history, direct observation, age, and clinical judgement before drawing any conclusion.

SUM

Screening Summary & Decision

Clinician consolidation worksheet · Scales 21–22

CONSOLIDATE & DECIDE

SCORE CONSOLIDATION (SCALE 21)

SCALE	SCORE	BAND	SETTING
01 · Parent Inattention	☐	☐	Home
02 · Parent Hyperactivity–Impulsivity	☐	☐	Home
03 · Teacher Observation	☐	☐	School
20 · Functional Impairment	☐	☐	All

CLINICAL IMPRESSION

DIFFERENTIAL FLAGS NOTED

NEXT-STEP DECISION (SCALE 22)

☐

Monitor & reassess

☐

Gather more input

☐

School support / plan

☐

Refer for full evaluation

CLINICIAN SIGNATURE

DATE



Professional use & disclaimer. The AbaTools ADHD Screening Pack provides screening–support instruments built around recognised ADHD symptom domains. It is intended for qualified professionals and does not constitute a diagnostic instrument, medical advice, or a substitute for comprehensive clinical evaluation. Diagnosis of ADHD requires a full assessment consistent with current clinical standards (e.g., DSM–5–TR), multi–informant data and the exclusion of alternative explanations. Use within your scope of practice and local regulations.